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Name : Mamatha(44Y/F)

Date : 23 Aug, 2026

Test Asked : Diabetic Profile Advanced A2381, G6pd

Report Status : Complete Report







Processed At :
5CA-711, 3rd Floor, HRBR 2nd
Block, Hennur, Bengaluru-560043

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Patient Name	: **MAMATHA (44Y/F)**	Tests Done	: **DIABETIC PROFILE ADVANCED A2381,G6PD**
Referred By	: **DR RAKESH H J**		
Address	: **(5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

26 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
DIABETIC PROFILE ADVANCED A2381	Ready
CHLORIDE	Ready
C-PEPTIDE	Ready
25-OH VITAMIN D (TOTAL)	Ready
FOLATE	Ready
FRUCTOSAMINE	Ready
HOMOCYSTEINE	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
INSULIN - FASTING	Ready
LIPASE	Ready
Lipoprotein (a) [Lp(a)]	Ready
LP-PLA2	Ready
SODIUM	Ready
URINARY MICROALBUMIN	Ready
VITAMIN B-12	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
ELEMENTS 22 (TOXIC AND NUTRIENTS)	Ready
IRON DEFICIENCY PROFILE	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready

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Address	: **(5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

26 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
APOLIPROTEIN RATIO	Ready
AMYLASE	Ready
BLOOD KETONE (D3HB)	Ready
GLUCOSE-6-PHOSPHATE DEHYDROGENASE	Ready

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Referred By	: **DR RAKESH H J**		
Address	: **(5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085**		

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
CARDIAC RISK MARKERS			
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	**3.3**	mg/L	< 3
HOMOCYSTEINE	**15.96**	µmol/L	<15
COMPLETE HEMOGRAM			
EOSINOPHILS	**0.9**	%	1-6
HEMATOCRIT(PCV)	**48.2**	%	36.0-46.0
HEMOGLOBIN	**15.3**	g/dL	12.0-15.0
LYMPHOCYTE	**47.9**	%	20-40
LYMPHOCYTES - ABSOLUTE COUNT	**3.55**	X 10^3 / µL	1.0-3.0
MEAN PLATELET VOLUME(MPV)	**13.8**	fL	6.5-12
PLATELET DISTRIBUTION WIDTH(PDW)	**16.2**	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	**52.5**	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	**48.9**	fL	39.0-46.0
TOTAL RBC	**5.25**	X 10^6/µL	3.8-4.8
DIABETES			
AVERAGE BLOOD GLUCOSE (ABG)	**166**	mg/dL	90-120
HbA1c	**7.4**	%	< 5.7
URINARY MICROALBUMIN	**25.8**	µg/mL	< 25
ELECTROLYTES			
SODIUM	**134.49**	mmol/L	136 - 145
IRON DEFICIENCY			
% TRANSFERRIN SATURATION	**12**	%	13 - 45
LIVER			
ALBUMIN - SERUM	**4.97**	gm/dL	3.2-4.8
PANCREATIC			
AMYLASE	**145.1**	U/L	28 - 100
LIPASE	**74.7**	U/L	5.0 - 60.0
RENAL			
EST. GLOMERULAR FILTRATION RATE (eGFR)	**82**	mL/min/1.73 m2	>= 90
URI. ALBUMIN/CREATININE RATIO (UA/C)	**32.7**	µg/mg of Creatinine	< 30
THYROID			
TOTAL THYROXINE (T4)	**4.4**	µg/dL	4.5-12
TSH - ULTRASENSITIVE	**11.9**	µIU/mL	0.54-5.30
VITAMIN			

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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Referred By	: **DR RAKESH H J**		
Address	: **(5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085**		

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
25-OH VITAMIN D (TOTAL)	**24.4**	ng/mL	30-100

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Patient Name	: MAMATHA (44Y/F)	**Sample Collected on (SCT)**	: 22 Aug 2026 16:26
Referred By	: DR RAKESH H J	**Sample Received on (SRT)**	: 23 Aug 2026 17:59
Address	: (5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085	**Report Released on (RRT)**	: 24 Aug 2026 06:17
		Sample Type | Barcode	: EDTA Whole Blood | GI875786

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**Colorimetry**	**15.3**	**g/dL**	**12.0-15.0**
Hematocrit (PCV)	**CPH Detection**	**48.2**	**%**	**36.0-46.0**
Total RBC	**HF & EI**	**5.25**	**X 10^6/µL**	**3.8-4.8**
Mean Corpuscular Volume (MCV)	Calculated	91.9	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	29	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.7	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**48.9**	**fL**	**39.0-46.0**
Red Cell Distribution Width (RDW - CV)	Calculated	13.9	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	243.3	-	*Refer Note below
MENTZER INDEX	Calculated	17.5	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.41	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	43.2	%	40-80
Lymphocytes Percentage	**Flow Cytometry**	**47.9**	**%**	**20-40**
Monocytes Percentage	Flow Cytometry	7.5	%	2-10
Eosinophils Percentage	**Flow Cytometry**	**0.9**	**%**	**1-6**
Basophils Percentage	Flow Cytometry	0.3	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	3.2	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	**Calculated**	**3.55**	**X 10^3 / µL**	**1.0-3.0**
Monocytes - Absolute Count	Calculated	0.56	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.02	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.07	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.01	X 10^3 / µL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	243	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	**Calculated**	**13.8**	**fL**	**6.5-12**
Platelet Distribution Width (PDW)	**Calculated**	**16.2**	**fL**	**9.6-15.2**
Platelet to Large Cell Ratio (PLCR)	**Calculated**	**52.5**	**%**	**19.7-42.4**
Plateletcrit (PCT)	Calculated	0.34	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF),ELEMENTS 22 (TOXIC AND NUTRIENTS),G6PD

Report Remarks : LabCode:2308045635/CT094

Dr Syeda Sumaiya
MD(Path)

Scan QR to verify(valid for 30 days from release time)

Patient Name	: MAMATHA (44Y/F)	**Sample Collected on (SCT)**	: 22 Aug 2026 16:26
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Address	: (5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085	**Report Released on (RRT)**	: 24 Aug 2026 06:17
		Sample Type | Barcode	: EDTA Whole Blood | GI875786

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	**H.P.L.C**	**7.4**	**%**

Bio. Ref. Interval. :

As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	**CALCULATED**	**166**	**mg/dL**

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF),ELEMENTS 22 (TOXIC AND NUTRIENTS),G6PD

Report Remarks : LabCode:2308045635/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD (Biochem)

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		Sample Type | Barcode	: **EDTA Whole Blood | GI875786**

TEST NAME	TECHNOLOGY	VALUE	UNITS
GLUCOSE-6-PHOSPHATE DEHYDROGENASE	PHOTOMETRY	9.5	U/g Hb

Bio. Ref. Interval. :-

7.0 - 20.5

Clinical Significance:
- The most common enzyme defect worldwide is a deficiency of glucose 6-phosphate dehydrogenase (G6PD). It is an X-linked recessive disorder - G6PD plays a key role in the generation of nicotinamide adenine dinucleotide phosphate (NADPH) which is critical for protection against oxidative stress
- Majority of individuals with this enzyme deficiency remain asymptomatic, in the absence of triggering factors like oxidative stress, bacterial / viral infections and after ingestion of certain drugs or fava beans.
- Symptoms can include neonatal jaundice (presents at 1-4 days of age) or acute hemolysis
- WHO defines classification of G6PD deficiency based on residual activity of the enzyme and level of hemolysis. - Reticulocytosis from any cause can mask some G6PD deficiency cases by raising the activity level. Comparison to other red blood cell enzyme activity levels may be useful.

Please correlate with clinical conditions.
Method:- ENZYMATIC COLORIMETRIC TEST

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF),ELEMENTS 22 (TOXIC AND NUTRIENTS),G6PD

Report Remarks : LabCode:2308045635/CT094

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MD(Path)

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Sample Type | Barcode : EDTA Whole Blood | GI875786

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ARSENIC	ICP-MS	0.42	µg/L	< 5
CADMIUM	ICP-MS	0.17	µg/L	< 1.5
MERCURY	ICP-MS	1	µg/L	< 5
LEAD	ICP-MS	44.58	µg/L	< 150
CHROMIUM	ICP-MS	3.88	µg/L	< 30
BARIUM	ICP-MS	2.1	µg/L	< 30
COBALT	ICP-MS	0.4	µg/L	0.10 - 1.50
CAESIUM	ICP-MS	1.79	µg/L	< 5
THALLIUM	ICP-MS	0.04	µg/L	< 1
URANIUM	ICP-MS	0.04	µg/L	< 1
STRONTIUM	ICP-MS	19.56	µg/L	8 - 38
ANTIMONY	ICP-MS	6.19	µg/L	0.10 - 18
TIN	ICP-MS	0.52	µg/L	< 2
MOLYBDENUM	ICP-MS	0.91	µg/L	0.70 - 4.0
SILVER	ICP-MS	1.16	µg/L	< 4
VANADIUM	ICP-MS	0.36	µg/L	< 0.8
BERYLLIUM	ICP-MS	0.06	µg/L	<0.10
BISMUTH	ICP-MS	0.49	µg/L	0.10 - 0.80
SELENIUM	ICP-MS	114.46	µg/L	60 - 340
ALUMINIUM	ICP-MS	11.85	µg/L	< 30
NICKEL	ICP-MS	4.92	µg/L	< 15
MANGANESE	ICP-MS	9.54	µg/L	7.10 - 20

Please correlate with clinical conditions.

Method :

ICP - MASS SPECTROMETRY
Note:Reference range has been obtained after considering 95% population as cutoff.

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF),ELEMENTS 22 (TOXIC AND NUTRIENTS),G6PD

Report Remarks : LabCode:2308045635/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD
(Biochem)

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Address	: (5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085	**Report Released on (RRT)**	: 24 Aug 2026 06:07
		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	161	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	52	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	96.07	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	117	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	3.1	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	2.24	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	1.8	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.54	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	109.1	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	23.32	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD
(Biochem)

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		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**E.C.L.I.A**	**24.4**	**ng/mL**
Bio. Ref. Interval. :-			

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.
Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD (Biochem)

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		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	490	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.
Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	103	ng/dL	80-200
TOTAL THYROXINE (T4)	**C.L.I.A**	**4.4**	**µg/dL**	**4.5-12**
TSH - ULTRASENSITIVE	**E.C.L.I.A**	**11.9**	**µIU/mL**	**0.54-5.30**

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Fully Automated Electrochemiluminescence Compititive Immunoassay
T4 - Competitive Chemi Luminescent Immuno Assay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Pregnancy reference ranges for TSH/USTSH :

Trimester	T3 (ng/dl)	T4 (µg/dl)	TSH/USTSH (µIU/ml)
1st	83.9-196.6	4.4-11.5	0.1-2.5
2nd	86.1-217.4	4.9-12.2	0.2-3.0
3rd	79.9-186	5.1-13.2	0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD (Biochem)

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Patient Name	: **MAMATHA (44Y/F)**	**Sample Collected on (SCT)**	: **22 Aug 2026 16:26**
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		Sample Type | Barcode	: **SERUM | FV678964**

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	14.19	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.89	mg/dL	0.55-1.02
BUN / Sr.CREATININE RATIO	CALCULATED	15.94	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	30.37	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	34.12	Ratio	< 52
CALCIUM	PHOTOMETRY	9.41	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	4.2	mg/dL	3.2 - 6.1

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	50	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170			
Method : Ferrozine method without deproteinization			
TOTAL IRON BINDING CAPACITY (TIBC)	PHOTOMETRY	400	µg/dL
Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl			
Method : Spectrophotometric Assay			
% TRANSFERRIN SATURATION	**CALCULATED**	**12**	**%**
Bio. Ref. Interval. : 13 - 45			
Method : Derived from IRON and TIBC values			
UNSAT.IRON-BINDING CAPACITY(UIBC)	PHOTOMETRY	350.2	µg/dL
Bio. Ref. Interval. : 162 - 368			
Method : SPECTROPHOTOMETRIC ASSAY			

Please correlate with clinical conditions.

Tests Done : DIABETIC PROFILE ADVANCED A2381

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM	**I.S.E - INDIRECT**	**134.49**	**mmol/L**

Bio. Ref. Interval. :
Adults: 136-145 mmol/l

Method : ION SELECTIVE ELECTRODE - INDIRECT

CHLORIDE	I.S.E - INDIRECT	105.24	mmol/L

Bio. Ref. Interval. :
ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).

Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

Tests Done : DIABETIC PROFILE ADVANCED A2381

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	74.4	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.32	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.05	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.27	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	25.8	U/L	< 38
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	17.1	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	13	U/L	< 34
SGOT / SGPT RATIO	CALCULATED	1.32	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	8.17	gm/dL	5.7-8.2
ALBUMIN - SERUM	**PHOTOMETRY**	**4.97**	**gm/dL**	**3.2-4.8**
SERUM GLOBULIN	CALCULATED	3.2	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.55	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg[1]method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
LP-PLA2	PHOTOMETRY	128	nmol/min/mL

Bio. Ref. Interval. :-

Low Risk : < 225 nmol/min/mL
High Risk : >= 225 nmol/min/mL

Clinical Significance:

Lp-PLA2, is an enzyme produced by inflammatory cells. It is predominantly associated with low-density lipoprotein (LDL) and high-density lipoprotein (HDL). Lp-PLA2 is a specific marker of vascular inflammation and found to be upregulated in atherosclerotic lesions especially in complex plaque prone to rupture. A meta-analysis found that Lp-PLA2 levels are positively correlated with increased risk of developing coronary heart disease and stroke. Lp-PLA2 is not an acute phase reactant and thus is unaffected by systemic inflammatory processes. Lp-PLA2 activity should be interpreted in conjunction with clinical evaluation and other risk factor assessment.

Specification:

Precision %CV : Intra assay %CV- 1.50% , Inter assay %CV-3.80%

Kit Validation Reference:

Alexander Thompson etal., The Lp-PLA2 Studies Collaboration (2010). "Lipoprotein-associated phospholipase A2 and risk of coronary disease, stroke, and mortality: collaborative analysis of 32 prospective studies". The Lancet 375 (9725): 1536-1544

Please correlate with clinical conditions.
Method:- ENZYMATIC ASSAY

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
Lipoprotein (a) [Lp(a)]	IMMUNOTURBIDIMETRY	19.8	mg/dL

Bio. Ref. Interval. :-

Adults : < 30.0 mg/dl

Clinical Significance:
Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:
Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:
Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.
Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
LIPASE	**PHOTOMETRY**	**74.7**	**U/L**

Bio. Ref. Interval. :-

Adults : 5.0 - 60.0 U/L

Interpretation:

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:
High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Specifications:
Precision: Intra assay (%CV): 3.35, Inter assay (%CV): 2.46, Sensitivity: 3.5 U/L.

Kit Validation References:
Tietz Nw Et Al. Lipase In Serum - The Elusive Enzyme: An Overview. Clin Chem 1993; 39:746-756.

Please correlate with clinical conditions.
Method:- ENZYMATIC COLORIMETRIC ASSAY

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
INSULIN - FASTING	C.L.I.A	8.87	µU/mL

Bio. Ref. Interval. :-

1.9-23 µU/mL

Clinical Significance

Type I (Insulin dependent: "Juvenile") diabetes is due to a destruction of the beta cells, with a consequence of absolute lack of insulin. In type II (Non insulin-dependent: "Maturity onset") diabetes, insulin resistance may play an important role; However after several years of evolution, beta-cells failure may occur, leading to a relative insulinopenia requiring, in some cases, insulin administration. Insulin resistance is associated with high circulation levels of the hormone.

For diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision: Intra Assay (%CV): 4.20 %, Inter Assay (%CV): 5.60%; Sensitivity: 0.03 µU/mL

External quality control program participation:

College Of American Pathologists: Insulin Survey (Ing): Cap Number: 7193855-01

Kit validation references:

Howanitz PJ, Howanitz JH, Henry JB. Carbohydrates.Clinical Diagnosis and Management by Laboratory Methods 1991 ;172-182.edited by Henry JB, Philadelphia, W.B Saunders Company.

Please correlate with clinical conditions.

Method:- One step Immunoenzymatic (Sandwich) assay.

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	**IMMUNOTURBIDIMETRY**	**3.3**	**mg/L**

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HOMOCYSTEINE	**PHOTOMETRY**	**15.96**	**µmol/L**

Bio. Ref. Interval. :-

Normal Levels : <15 µmol/L
Mild Hyperhomocysteinemia : 15-30 µmol/L
Moderate Hyperhomocysteinemia : 30-100 µmol/L
Severe Hyperhomocysteinemia : >100 µmol/L

Clinical Significance:
Homocysteine is linked to increased risk of premature coronary artery disease, stroke and thromboembolism. Moreover, alzheimers disease, osteoporosis, venous thrombosis, schizophrenia, cognitive deficiency and pregnancy complications also elevates Homocysteine levels. The results should be interpreted in conjunction with clinical history and other findings.

High Values:
Elevated homocysteine levels might be due to increasing age, genetic traits, drugs, renal dysfunction and dietary deficiency of vitamins or smoking. To lower your homocysteine, eat more green vegetables, stop smoking, alcohol. Folic acid helps lowering elevated levels.

Specifications:
Kit Validation Reference:
Eikelboom JW, et al Ann Intern Med 131 : 363-75 (1999)
https://www.healthline.com/health/homocysteine-levels

Please correlate with clinical conditions.
Method:- SMALL MOLECULE CAPTURE TECHNOLOGY (SMT)

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
FRUCTOSAMINE	PHOTOMETRY	269.1	µmol/L

Bio. Ref. Interval. :-

Normal < 286 µmol/L

Clinical Significance:

Fructosamine assay is useful in monitoring the degree of glycemia over short-to-intermediate time frames (1-3 weeks) concentration greater than the established normal range is an indication of prolonged hyperglycemia of 1-3 weeks or longer. The higher fructosamine value, poorer is the degree of glycemia control.

Specifications:

Precision %CV : Intra assay %CV- 3.2% , Inter assay %CV-4.0%, Sensitivity:- 290 umol/L

Kit Validation Reference:

Howey JEA, Browning MCK, Fraser CG. Assay of serum fructosamine that minimizes standardization and matrix problems: Use to assess components of biological va-riation. Clin Chem 1987; 33: 269- 272.

Please correlate with clinical conditions.

Method:- NITROBLUE TETRAZOLIUM ASSAY (NBT)

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
FOLATE	C.L.I.A	14.2	ng/mL

Bio. Ref. Interval. :
> 5.38 ng/ml

Clinical Significance: Low folate intake, malabsorption as a result of gastrointestinal diseases, pregnancy, and drugs such as phenytoin are causes of folate deficiency.3 Folate deficiency is also associated with chronic alcoholism. Serum folate measurement provides an early index of folate status.

Specifications: Precision: Intra assay (%CV): 7.93, Inter assay (%CV): 7.19, Sensitivity: 0.35 ng/mL.

Kit Validation References: Steinkamp RC. Vitamin B12 and folic acid: clinical and pathophysiological considerations. In: Brewster MA, Naito HK, eds. Nutritional Elements and Clinical Biochemistry. New York: Plenum Publishing Corp.; 1980:169–240

Method : COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

Please correlate with clinical conditions.

Tests Done : DIABETIC PROFILE ADVANCED A2381

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	**CALCULATED**	**82**	**mL/min/1.73 m2**

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- 2021 CKD EPI Creatinine Equation

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD (Biochem)

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Patient Name	: MAMATHA (44Y/F)	**Sample Collected on (SCT)**	: 22 Aug 2026 16:26
Referred By	: DR RAKESH H J	**Sample Received on (SRT)**	: 23 Aug 2026 18:01
Address	: (5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085	**Report Released on (RRT)**	: 24 Aug 2026 06:07
		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS
C-PEPTIDE	E.C.L.I.A	3.88	ng/mL

Bio. Ref. Interval. :-

1.10 – 4.40 ng/ml

Clinical Significance
C-peptide, a polypeptide consisting of 31 amino acids (MW~3000), is stored in the secretory granules of the beta cells and released into circulation in equimolar amounts with insulin. The determination of C-peptide provides an assessment of endogenous insulin secretory reserves in patients with diabetes mellitus and is considered a more reliable indicator of insulin secretion than insulin itself. The primary indication for measuring C-peptide is for the evaluation of fasting hypoglycemia. It is also used to monitor patient's response to pancreatic surgery. C-peptide levels increase in insulinomas and beta-cell tumors.

Specifications: Precision: Intra assay (%CV): 2.9%, Inter assay (%CV): 3.6%; Sensitivity: 0.02 ng/ml

Kit Validation reference:
Clerk PM, Assays for insulin, proinsulin (s) and c-peptide. Ann clin biochem 1999;36(5):541-564

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED ELECTROCHEMILUMINESCENCE IMMUNOASSAY

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD (Biochem)

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		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS
BLOOD KETONE (D3HB)	PHOTOMETRY	0.4	mg/dL

Bio. Ref. Interval. :-

0.21-2.81 mg/dL

Clinical Significance:
Three types of ketones can be produced in body D-3- Hydroxybutyrate, Acetoacetate and Acetone. D-3- Hydroxybutyrate accounts for approximately 75% of the ketone bodies. During periods of ketosis, D-3- Hydroxybutyrate increases more than the other two. It has been shown to be a better index of ketoacidosis. In diabetics, D-3- Hydroxybutyrate is needed for the assessment of the severity of diabetic coma and to calculate insulin requirements.

Speficcation:
Precision: Intra assay (%CV): 4.53, Inter assay (%CV): 2.9, Sensitivity: 10.41 mg/dL.

Kit validation references:
Mcmurray, C.H., Blanchflower, W.J., Rice, D.A., ClinChem., 1984;30:No.3.

Please correlate with clinical conditions.
Method:- ENZYMATIC (KINETIC)

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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		Sample Type | Barcode	: SERUM | FV678964

TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETRY	121	mg/dL

Bio. Ref. Interval. :
Male : 86 - 152
Female : 94 - 162

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETRY	89	mg/dL

Bio. Ref. Interval. :
Male : 56 - 145
Female : 53 - 138

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APO B / APO A1 RATIO (APO B/A1)	CALCULATED	0.7	Ratio

Bio. Ref. Interval. :

Male : 0.40 - 1.26
Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

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TEST NAME	TECHNOLOGY	VALUE	UNITS
AMYLASE	**PHOTOMETRY**	**145.1**	**U/L**

Bio. Ref. Interval. :-

Adults : 28-100 U/L

Interpretation:
Lipemic Sera (Hypertriglyceridemia) may contain inhibitors, Which falsely depress results. About 20% of patients with Acute Pancreatitis have abnormal lipids. Normal serum amylase may occur in Pancreatitis, Especially relapsing and chronic pancreatitis. Moderate increases may be reported in normal pregnancy.

Clinical Significance:
Causes of high Serum Amylase include Acute Pancreatitis, Pancreatic Pseudocyst, Pancreatic Ascites, Pancreatic Abscess, Neoplasm in or adjacent to Pancreas, Trauma to Pancreas, and common Duct Stones. Nonpancreatic Causes include inflammatory salivary lesions (Eg, Mumps), Perforated Peptic Ulcer, Intestinal Obstruction, Biliary Tract Disease, Peritonitis, Acute Appendicitis, Diabetic Ketoacidosis, and Extrapancreatic Carcinomas. Amylase levels more than 25-fold the upper limit of normal are often found when metastatic tumors produce Ectopic Amylase.

Specifications:
Precision: Intra assay (%CV): 2.82, Inter assay (%CV): 2.49, Sensitivity: 10.9 U/L.

Kit Validation References:
Rauscher, E., et coll., Fresenius Z. Analyt. Chem. 324 (1986) 304-305.

Please correlate with clinical conditions.
Method:- ENZYMATIC COLORIMETRIC TEST

Tests Done : DIABETIC PROFILE ADVANCED A2381

Report Remarks : LabCode:2308045706/CT094

M. Anuswaru

Dr.Anuswaru MBBS,MD (Biochem)

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Patient Name	: MAMATHA (44Y/F)	**Sample Collected on (SCT)**	: 22 Aug 2026 16:26
Referred By	: DR RAKESH H J	**Sample Received on (SRT)**	: 23 Aug 2026 16:34
Address	: (5600856596),Dr Heal Pain Cure HospitA,No 208 209, 100 feet, Outer Ring Rd, 4th Block, Banashankari 3rd Stage, Bengaluru, Karnataka 560085.,560085	**Report Released on (RRT)**	: 23 Aug 2026 18:21
		Sample Type | Barcode	: URINE | FX773865

TEST NAME	TECHNOLOGY	VALUE	UNITS
CREATININE - URINE	PHOTOMETRY	79	mg/dL

Bio. Ref. Interval. :
Male: 39 - 259 mg/dl
Female: 28 - 217 mg/dl

Method : Creatinine Jaffe Method, Rate-Blanked and Compensated

Please correlate with clinical conditions.

Tests Done : URINARY MICROALBUMIN

Report Remarks : LabCode:2308108764/CT094

Dr Yashashwini
MD(Path)

Dr Priya Tripathi
MD(Path)

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(BDA), No 1159, Bangalore

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		Sample Type | Barcode	: URINE | FX773865

TEST NAME	TECHNOLOGY	VALUE	UNITS
DIABETES SCREEN (URINE)			
URINARY MICROALBUMIN	**PHOTOMETRY**	**25.8**	**µg/mL**
Bio. Ref. Interval. : Adults: Less than 25 µg/ml			
Method : Fully Automated Immuno Turbidometry			
URI. ALBUMIN/CREATININE RATIO (UA/C)	**CALCULATED**	**32.7**	**µg/mg of Creatinine**
Bio. Ref. Interval. : Adults : Less than 30 µg/mg of Creatinine			
Method : Derived from Albumin and Creatinine values			

Please correlate with clinical conditions.

Tests Done : URINARY MICROALBUMIN

Report Remarks : LabCode:2308108764/CT094

Dr Yashashwini
MD(Path)

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MD(Path)

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CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient, as the reported results are dependent on individual assay method(s) used/ equipments used and quality of specimen received.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume:
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- Thyrocare Discovery video link:- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at customersupport@thyrocare.com or call us on 022-3090 0000.

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*As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited